

DEEP VEIN THROMBOSIS (DVT)

A BLOOD CLOT THAT DEVELOPES IN THE DEEPER VEINS – USUALLY IN THE LEG/KNEE/HIP AREA

SIGNS & SYMPTOMS (S&S):

- Edema
- Redness
- Warmth
- Pain / Tenderness
- Severity depends on size/location of vein occlusion

DIAGNOSTIC TESTING:

- LABS – CBC, PT, PTT, INR, ↑ D-dimer, CRP, Factor V, Protein S&C Syndrome, Genetic Studies
- Duplex Ultrasonography (US) – Vein is larger than normal & thrombus is incompressible

FOCUS ON PREVENTION:

- Ambulation
- Staying well-hydrated
- Foot pump/flex when sedentary (incl./esp. travel time)

DVT NURSING INTERVENTIONS:

- Close monitoring
- Bleeding Assmnt & Risk Mgmt
- Manage pain & anxiety
- Patient & Family Education

Don't Massage!!

Elevate

Ensure Bedrest

Pharmacologic Intervention

Heparin – Stop the platelet party

Anticoagulants – Stop the factor frenzy

Remember “DEEP”

RISK FACTORS & VIRCHOW'S TRIAD:

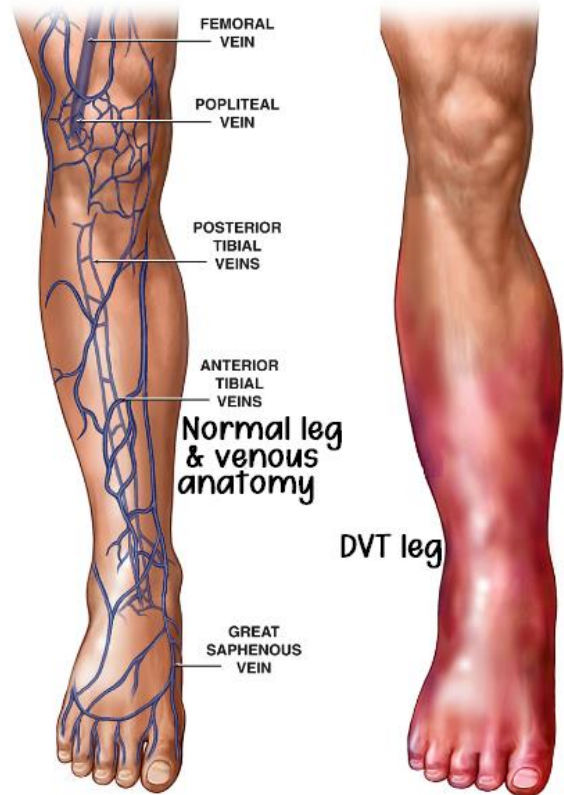
Endothelial Injury + Hypercoagulable State
+ Venous Stasis = BLOOD CLOT

OK, BUT WHAT CAUSES THESE THINGS?

Endothelial Damage: Trauma to injury (incl venipuncture); indwelling devices (stents, valves, IV access); Medications; IV drug usage

Venous Stasis – circulation is impaired due to: Immobilization (travelling long hours); Varicose veins; Surgery (esp hip or knee); Obstruction (late pregnancy, Obesity); Heart Failure; A-fib

Hypercoagulable State – Dehydration; Oral birth control use; Post-partum; Severe illness (Sepsis); HIT (Heparin Induced Thrombocytopenia)



COLLABORATIVE CARE:

- Heparin (unfractionated or LMWH) infusion or SQ injections
- Oral Anticoagulation
- Monitoring for therapeutic anticoagulation
- Possible CT of Chest to r/o PE

PATHOPHYSIOLOGY OF A BLOOD CLOT:

Intravascular Endothelial Injury > Activated Platelets gather > Clotting factors are released > Fibrin is created & comes in to hold everything together > More blood cells show up to “help” but keep collecting & a clot is formed & keeps growing larger.

platelets & fibrin

